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Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

PRESIDENTIAL EBOLA TASK FORCE 17

MVE17 Incident Management System



## Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°173/MVE-BDBV/03/11/2026

Reporting date: November 3, 2026 Publication date: November 4, 2026

### HIGHLIGHTS (last 24 hours)

**Over the past 24 hours, sixty-two (62) new confirmed cases and 34 confirmed deaths, including 18 community deaths and 16 deaths within treatment centers, have been reported. The new cases were recorded in North Kivu (36), Ituri (23), and Haut-Uélé (3). A total of 441 new samples were analyzed in the laboratories that submitted their data; 62 of these samples tested positive for the Bundibugyo virus, representing a positivity rate of 14.1%. No new health zones were affected in the past 24 hours. In Bas-Uélé, all contacts have completed their follow-up period. However, six (6) contacts have been lost to follow-up for more than 27 days, and their final status has not yet been determined.**

| ACCUMULATIVE CASE | ACCUMULATIVE DEATH CONFIRMED | LETHALITY | PATIENTS IN ISOLATION/CTE | ACCUMULATIVE HEALED | TRACKING RATES CONTACTS (Of the Day) |
|---|---|---|---|---|---|
| 10668 | 5198 | 48.7% | 1056 | 2,881 | 83.9% |

### Affected provinces and affected health zones

**8 Provinces affected**

**Haut-Uélé, Ituri, North Kivu, South Kivu, Tshopo, Bas-Uélé, South Ubangi, and North Ubangi**

**67 out of 178 health zones affected (37.6%)**

### Summary of key points from the last 24 hours

On November 3, 2026, sixty-two (62) new confirmed cases of Ebola virus disease (EVD) were reported in three provinces: North Kivu (36 cases), Ituri (23 cases), and Haut-Uélé (3 cases). Among these cases, 18 community deaths were reported (10 in North Kivu, 7 in Ituri, and 1 in Haut-Uélé). In addition, 16 confirmed deaths were recorded in Ebola Treatment Centers (9 in Ituri, 4 in North Kivu, 2 in Haut-Uélé, and 1 in Tshopo), bringing the total number of confirmed deaths reported during this period to 34.

In addition, 22 patients (16 in Ituri, 3 in North Kivu, 2 in Tshopo and 1 in Haut-Uélé) were declared cured after obtaining two negative results in the control test for Ebola virus disease.

Since the start of the epidemic, the DRC has recorded 10,668 confirmed cases, including 5,198 deaths, representing an overall case fatality rate of 48.7%. Ituri remains the epicenter of the epidemic, accounting for 70.2% of cumulative confirmed cases and 37.1% of new reported cases, while North Kivu represents 58.1% of new cases and has a case fatality rate of 54.8%, higher than the national average.

No new health zones have been affected in the last 24 hours.

Thus, 67 out of 178 health zones (37.6%), spread across eight provinces, have been affected since the start of the epidemic, namely: Turi, North Kivu, Haut-Uélé, Tshopo, South Kivu, Bas-Uélé, South Ubangi and North Ubangi.

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**Table 1. Distribution of confirmed cases and deaths by affected province, as of November 3, 2026.**

| Province | New confirmed cases (24h) | Confirmed cases | Deaths (confirmed) | Lethality | Affected health zones |
|---|---|---|---|---|---|
| Ituri | 23 | 7485 | 3,532 | 47.2% | 28/36 (77.8%) |
| North Kivu | 36 | 2593 | 1,422 | 54.8% | 17/34 (50.0%) |
| Haut-Uélé | 3 | 470 | 208 | 44.3% | 7/13 (53.8%) |
| Tshopo | 0 | 94 | 29 | 30.9% | 7/23 (30.4%) |
| South Kivu | 0 | 3 | 1 | 33.3% | 1/34 (2.9%) |
| Bas-Uélé | 0 | 13 | 4 | 30.8% | 4/11 (36.4%) |
| South Ubangi | 0 | 2 | 1 | 50.0% | 1/16 (6.3%) |
| North Ubangi | 0 | 8 | 1 | 12.5% | 2/11 (18.2%) |
| **Total** | **62** | **10,668** | **5,198** | **48.7%** | **67/178 (37.6%)** |

**Table 2. Distribution of confirmed cases and deaths by province and zone as of November 3, 2026**

| Province / Health Zone | Cumulative number | | | Current situation (24 hours) | | | |
|---|---|---|---|---|---|---|---|
| | Case (n) | Deaths (n) | Lethality | New Case | Community deaths (Swab+) | Confirmed deaths within the CTE | Total confirmed deaths |
| **Ituri** | **7485** | **3532** | **47.2%** | **23** | **7** | **9** | **16** |
| Adja | 11 | 1 | 9.1% | | | | 0 |
| Aru | 25 | 7 | 28.0% | | | | 0 |
| Ariwara | 8 | 2 | 25.0% | | | | 0 |
| Aungba | 12 | 5 | 41.7% | | | | 0 |
| Bamboo | 220 | 50 | 22.7% | | | | 0 |
| Boga | 2 | 2 | 100.0% | | | | 0 |
| Bunia | 2019 | 707 | 35.0% | 3 | 1 | 1 | 2 |
| Damascus | 41 | 10 | 24.4% | | | | 0 |
| Drodro | 15 | 7 | 46.7% | | | | 0 |
| Fataki | 84 | 31 | 36.9% | | | | 0 |
| Gethy | 11 | 7 | 63.6% | | | | 0 |
| Kambala | 2 | 0 | 0.0% | | | | 0 |
| Kilo | 40 | 13 | 32.5% | | | | 0 |
| Komanda | 193 | 119 | 61.7% | | | | 0 |
| Lita | 333 | 167 | 50.2% | 0 | 0 | 1 | 1 |
| Logo | 14 | 9 | 64.3% | | | | 0 |
| Lolwa | 37 | 11 | 29.7% | | | | 0 |
| Mahagi | 5 | 1 | 20.0% | | | | 0 |
| Mambasa | 118 | 65 | 55.1% | 5 | 3 | 1 | 4 |

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|---|---|---|---|---|---|---|---|
| Mandima | 511 | 277 | 54.2% | 9 | 2 | 3 | 5 |
| Mangala | 402 | 172 | 42.8% | 4 | 0 | 0 | 0 |
| Mongbwalu | 726 | 352 | 48.5% | 0 | 0 | 1 | 1 |
| Nia-Nia | 343 | 198 | 57.7% | | | | 0 |
| Nizi | 855 | 313 | 36.6% | 0 | 0 | 1 | 1 |
| Nyankunde | 140 | 43 | 30.7% | | | | 0 |
| Rimba | 26 | 7 | 26.9% | 1 | 1 | 0 | 1 |
| Rwampara | 1206 | 436 | 36.2% | 1 | 0 | 1 | 1 |
| Tchomia | 86 | 27 | 31.4% | | | | 0 |
| To ventilate | N / A | 493 | N / A | | | | 0 |
| **North Kivu** | **2593** | **1422** | **54.8%** | **36** | **10** | **4** | **14** |
| Blessed | 737 | 485 | 65.8% | | | | 0 |
| Good | 48 | 19 | 39.6% | 22 | 9 | 1 | 10 |
| Butembo | 346 | 202 | 58.4% | 1 | 0 | 0 | 0 |
| Goma | 1 | 0 | 0.0% | | | | 0 |
| Kalunguta | 103 | 44 | 42.7% | 2 | 0 | 0 | 0 |
| Katwa | 682 | 421 | 61.7% | 3 | 1 | 3 | 4 |
| Kayna | 3 | 2 | 66.7% | | | | 0 |
| Kyondo | 46 | 22 | 47.8% | | | | 0 |
| Lubero | 43 | 15 | 34.9% | | | | 0 |
| Mabalako | 120 | 39 | 32.5% | 2 | 0 | 0 | 0 |
| Alimbongo | 6 | 4 | 66.7% | | | | 0 |
| Manguredjipa | 16 | 5 | 31.3% | | | | 0 |
| Masereka | 45 | 13 | 28.9% | 1 | 0 | 0 | 0 |
| Musienene | 162 | 77 | 47.5% | | | | 0 |
| Mutwanga | 77 | 16 | 20.8% | | | | 0 |
| Oicha | 100 | 38 | 38.0% | 3 | 0 | 0 | 0 |
| Vuhovi | 58 | 20 | 34.5% | 2 | 0 | 0 | 0 |
| **Upper Uele** | **470** | **208** | **44.3%** | **3** | **1** | **2** | **3** |
| Boma Mangbetu | 46 | 19 | 41.3% | | | | 0 |
| Dungu | 1 | 1 | 100.0% | | | | 0 |
| Gombari | 3 | 1 | 33.3% | | | | 0 |
| Isiro | 117 | 56 | 47.9% | 2 | 1 | 2 | 3 |
| Pawa | 116 | 65 | 56.0% | | | | 0 |
| Rungu | 9 | 5 | 55.6% | | | | 0 |
| Wamba | 178 | 61 | 34.3% | 1 | 0 | 0 | 0 |
| **Tshopo** | **94** | **29** | **30.9%** | 0 | 0 | 1 | 1 |
| Bafwasende | 18 | 3 | 16.7% | | | | 0 |
| Kabondo | 22 | 4 | 18.2% | 0 | 0 | 1 | 1 |
| Lubunga | 1 | 0 | 0.0% | | | | 0 |
| Makiso - Kisangani | 37 | 14 | 37.8% | | | | 0 |

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|---|---|---|---|---|---|---|---|
| Mangobo | 10 | 7 | 70.0% | | | | 0 |
| Tshopo | 5 | 0 | 0.0% | | | | 0 |
| Wanie-Rukula | 1 | 1 | 100.0% | | | | 0 |
| **South Kivu** | **3** | **1** | **33.3%** | 0 | 0 | 0 | 0 |
| Miti-Murhesa | 3 | 1 | 33.3% | | | | 0 |
| **Bas Uélé** | **13** | **4** | **30.8%** | 0 | 0 | 0 | 0 |
| Buta | 1 | 1 | 100.0% | | | | 0 |
| Viadana | 6 | 1 | 16.7% | | | | 0 |
| Dakwa | 1 | 0 | 0.0% | | | | 0 |
| Ganga | 5 | 2 | 40.0% | | | | 0 |
| **South Ubangi** | **2** | **1** | **50.0%** | 0 | 0 | 0 | 0 |
| Bulu | 2 | 1 | 50.0% | | | | 0 |
| **North Ubangi** | **8** | **1** | **12.5%** | 0 | 0 | 0 | 0 |
| Wapinda | 5 | 1 | 20.0% | | | | 0 |
| Yakoma | 3 | 0 | 0.0% | | | | 0 |
| **Total** | **10668** | **5198** | **48.7%** | **62** | **18** | **16** | **34** |

***These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.**

**NA: Not Applicable.**

## Table 3. Status of alerts notified by province for November 3, 2026

| DPS | Alerts Received | | Total alerts | Alerts verified and validated (suspected case) | | Alerts verified and invalidated (no suspected cases) | | Number of suspects investigated ed | Number of suspects No investigated ed | Number of suspects investigated and transferred At CT/CTE (of the day) |
|---|---|---|---|---|---|---|---|---|---|---|
| | Living | Deceased | | Living | Deceased | Living | Deceased | | | |
| Haut-Uélé | 179 | 5 | 184 | 11 | 2 | 168 | 3 | 13 | 0 | 3 |
| Bas Uélé | 165 | 3 | 168 | 7 | 3 | 158 | 0 | 10 | 0 | 0 |
| Ituri | 1,292 | 102 | 1,394 | 132 | 98 | 1,160 | 4 | 230 | 0 | 101 |
| North Kivu | 1942 | 80 | 2022 | 160 | 78 | 1,782 | 2 | 238 | 0 | 122 |
| South Ubangi | ND | ND | ND | ND | ND | ND | ND | ND | ND | ND |
| North Ubangi | ND | ND | ND | ND | ND | ND | ND | ND | ND | ND |
| Tshopo | 129 | 1 | 130 | 10 | 1 | 119 | 0 | 11 | 0 | 1 |
| **Total General** | **3,707** | **191** | **3,898** | **320** | **182** | **3,387** | **9** | **502** | **0** | **227** |

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**Figure 1. Evolution of cases by date of symptom onset over the past 21 days**

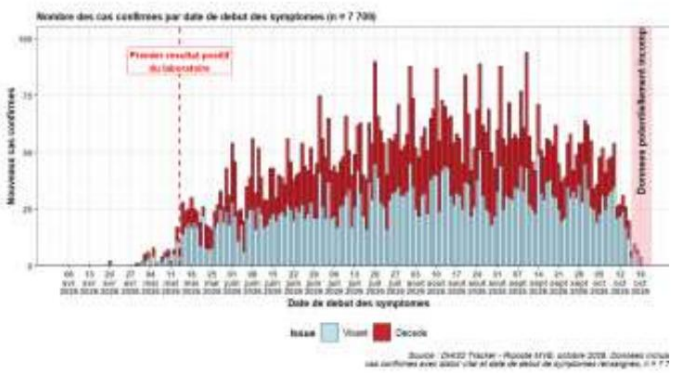
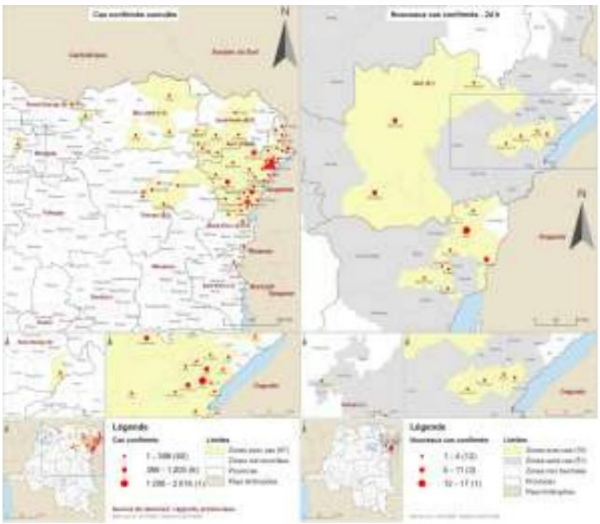


**Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.**



## 1. COORDINATION

### 1.1. Main Actions

• Support for response activities in the provinces **2. RESPONSE ACTIONS BY PILLAR**

### 2.1. Epidemiological surveillance

### 2.1.1. Main Actions

As of November 3, 2026, 3,898 alerts had been reported in the five provinces that submitted their data (Bas-Uélé, Haut-Uélé, Ituri, Nord-Kivu, and Tshopo), including 3,707 alerts of living persons and 191 of deaths; all were verified (100%). Of these, 502 (12.9%) were validated as suspected cases and 3,396 (87.1%) were invalidated. All 502 validated suspected cases were investigated (100%), and 227 were transferred to COVID-19 treatment centers (Table 3).

In total, 39,667 contacts were under follow-up in the five provinces with contacts in follow-up; 33,281 were seen in the last 24 hours, representing a follow-up rate of 83.9%, below the required threshold of 95.0% and slightly down from the previous day (84.4%) (Table 4). By province, the follow-up rate is 97.3% in South Ubangi (109/112), 88.9% in Ituri (15,704/17,670), 84.8% in Haut-Uélé (1,960/2,311), including 53.9% in Boma Mangbetu (146/271) and 74.7% in Wamba (522/699), 79.6% in North Kivu (15,179/19,070), including 65.3% in Beni (5,008/7,670), none of the 141 contacts in Kayna or the 301 contacts in Lubero having been seen, and 65.3% in Tshopo (329/504).

**Table 4. Status of contact tracing by province, as of November 3, 2026**

| Province | Contacts under follow-up | Contacts viewed | tracking rate |
|---|---|---|---|
| Ituri | 17,670 | 15,704 | 88.9% |
| Haut-Uélé | 2,311 | 1960 | 84.8% |
| North Kivu | 19,070 | 15,179 | 79.6% |
| Tshopo | 504 | 329 | 65.3% |
| Bas-Uélé | N / A | N / A | N / A |
| South Ubangi | 112 | 109 | 97.3% |

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| | | | |
|---|---|---|---|
| North Ubangi | 0 | 0 | N / A |
| Total | **39,667** | **33,281** | **83.9%** |

## 2.2. Update on checkpoint and point of entry (PoE/PoC) activities

### 2.2.1. Main actions

For the day of November 3, 2026, six (6) alerts were notified to the PoE/PoC, including three (3) in Ituri, one (1) in North Kivu, one (1) in South Kivu and one (1) in Bas-Uélé: in Ituri, an 18-year-old female patient with a fever (38 °C) at the Biakato Mayi PoC (Mandima), validated, transferred to the ESS and sampled, a 42-year-old man at the same PoC, validated but who refused his transfer, and the lifeless body of a 19-year-old man intercepted at the Mudzi Bala PoC (Bunia) and sampled; in North Kivu, a live alert notified at the Barrière Mavivi PoC (Beni) and invalidated after investigation; In South Kivu, the body of a 77-year-old woman who died in Bujumbura and was repatriated via the Kavimvira PoE to Nundu (Fizi) was reported without any samples being taken; in Bas-Uélé, the lifeless body of a 26-year-old man was intercepted at the PK25 Koteli PoC and taken to the morgue. Thus, 1 out of 2 validated live alerts was transferred (50%), 5 out of 6 alerts were investigated within 24 hours (83%), and 2 out of 3 bodies were taken or secured (67%) (Table 5). In total, 260,995 travelers passed through the PoE/PoCs of the five provinces listed in Table 5, of whom 97% were screened. Outside the table, Haut-Uélé recorded 21,398 travellers (11 out of 12 PoCs) and Sud-Ubangi 2,622 (3 out of 5 posts), without any alerts notified.

**Table 5. Status of key PoE/PoC indicators by province, as of November 3, 2026**

| Indicators | Ituri | North-Kivu | South Kivu | Tshopo | Bas-Uele | Overall |
|---|---|---|---|---|---|---|
| Report completeness | 97% (60/62) | 86% (18/21) | 56% (5/9) | 100% (18/18) | 59% (13/22) | 86.4% (114/132) |
| Strategic PoC/PoE | 1 | N / A | N / A | N / A | N / A | 1 |
| Number of past travelers | 143,881 | 82,100 | 6,600 | | 25,363 | 3051 | 260,995 |
| Proportion of travelers screened | 96% | 98% | 99% | 99% | 98% | 97% |
| % of travelers who washed their hands | 96% | 98% | 99% | 99% | 99% | 97% |
| % of those travelers made aware | 99% | 98% | 100% | 100% | 73% | 99% |
| Number of alerts notified | 03 | 01 | 01 | 00 | 01 | 6 |
| Number of validated live alerts | 02 | 00 | 00 | 00 | 00 | 2 |
| Number of lifeless bodies intercepted | 01 | 00 | 01 | 00 | 01 | 3 |
| % of suspected cases transferred to CT/CTE | 50% (1/2) | 0% | 0% | 0% | 0% | 50% (1/2) |
| % of lifeless bodies swabed/secured | 100% (1/1) | 0% | 0% (0/1) | 0% | 100% (1/1) | 67% (2/3) |
| % of suspected cases investigated within 24 hours | 100% (3/3) | 100% (1/1) | 0% (0/1) | 0% | 100% (1/1) | 83% (5/6) |

#### 2.2.2. Challenges

**In Bas-Uélé,** coverage is only 59% (13/22): 5 PoCs in the Viadana health zone did not function due to the service provider strike, and 4 did not report due to lack of connectivity and mobility; only 73% of travelers were made aware of the service, and coverage remains at 22 out of 61 sites (36%). In South Kivu, coverage remains low (56%; 5/9), the antennas at the Baraka station are not reporting, and the PoEs at Kavimvira, Kalundu Port, and
Baraka still do not have an isolation facility; in Ituri, 5,328 travellers (3.7%) refused screening, with high refusal rates in Pluto (48%), Nzebi (47%), Frigo (40%) and Pont Shari (27%), the Avakubi PoC is not functional due to insecurity, 4 PoCs (3 in Nia-Nia and 1 in Mandima)

### 2.3. Laboratory

### 2.3.1. Main actions

- **Ituri:** 23 new positive results (16 living and 7 dead) out of 202 new samples analyzed (positivity: 11.4%).

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North **Kivu:** 36 new confirmed cases (26 living and 10 deaths) identified through surveillance; laboratories 202 samples (17.8%) were analyzed from Beni and Butembo.

• Tshopo: 8 new samples analyzed, all revealed negative • Haut-Uélé: 3 new positive results (2 living, in Isiro and Wamba, and 1 death, the newborn of a recovered parturient who died at the CTE in Isiro) out of 17 new samples analyzed at the Isiro laboratory (positivity: 16.7%).

• **Bas-Uélé :** Four new samples (three blood and one swab) from Buta and Likati were received at the Buta laboratory, but none could be analyzed due to a generator failure; furthermore, 12 samples from Viadana analyzed at the Isiro laboratory all came back negative. No laboratory data was transmitted by North Ubangi and South Ubangi.

## 2.4. Infection Prevention and Control (IPC/EDS)

### 2.4.1. Main actions

• **Ituri :** 25 rings open out of 38 (65.8%), 25 households decontaminated out of 37 (67.6%) and 10 out of 10 FOSA (100%); 83 EDS carried out out of 115 alerts, including 6 postponements (72.2%), and 75 bodies swabés; the CREC facilitated the decontamination of 26 out of 26 households (100%), including 2 structures, and of 2 out of 2 ESS, the completion of 30 out of 30 EDS and the securing of 3 bodies; the teams remain insufficient in Mangala, Mandima, Damas and Mambasa.

• **North Kivu :** 9 rings open out of 15 (60.0%), 16 households decontaminated out of 29 (55.2%) and 22 out of 26 ESS (84.6%), none of the 16 decontaminated households having been provided with kits; training and capacity building: 356 service providers briefed, 165 ESS monitoring and support and 10 PCI-WASH score card assessments in 10 ESS (1 in Mabalako, 3 in Katwa, 1 in Beni, 2 in Butembo and 3 in Lubero; average score: 61.8%); 227 DLM functionality monitoring and 8 triage units installed (3 in Butembo, 2 in Musienene and 3 in Kyondo); risk assessment of exposure of 17 PPL, including 7 at high risk (2 out of 9 in Beni and 5 out of 8 in Oicha) 15 schools being supplied with PCI-WASH kits by NRC in Mabalako, 2 schools assessed and 11 public places decontaminated in Kalunguta; 56 EDS carried out out of 81 alerts, including 22 postponements (69.1%), 53 bodies swabed and 23 postponements to the following day.

• **Tshopo :** 1 ring opened in Kabondo and decontamination of the isolation room and emergency room of the Kabondo HGR around the 94th confirmed case; assessment of 4 out of 7 PPLs around the 93rd case (including 2 at high risk) and of 6 out of 9 around the 94th case; training and capacity building: start of training of 60 PPLs from DHKIS on IPC before its reopening, briefing of supervisors of the Kabondo and Mangobo health zones on electronic tools (monitoring of ESS, assessment of PPLs, scorecard and dashboard), monitoring and support of 8 out of 8 ESSs in 3 health zones with 182 providers briefed on standard precautions (100%) and reassessment of the Alabul health center (scorecard: 43.5% against 40.3% at the initial assessment).

• **Haut-Uélé :** briefing of ambulance drivers and drivers of EDS teams on compliance with IPC measures; training and capacity building: 29 service providers briefed (11 in Isiro, including 2 nurses, 4 laboratory technicians and 5 hygienists, 9 nurses in Wamba and 9 in Pawa, including 1 doctor and 8 hygienists) on the wearing and removal of PPE, waste management, bio-cleaning and hand hygiene; 8 ESS visited in 7 AS, of which 3 were assessed (Libiki Clinic: 42%; Matyogo Health Center: 46%; Bayenga Hospital: 72.6%); 2 functional triage units (Wamba Clinic and Matari Clinic) and 19 people sensitized to barrier measures in Boma Mangbetu; no ring opened nor household decontaminated around the 3 new cases.

• **Bas-Uélé :** monitoring and support of 12 out of 12 ESS in the health zones of Buta, Ganga and Viadana; training and capacity building: briefing of 32 out of 32 service providers and 7 out of 7 hygienists (100%) on the mandatory wearing of PPE and standard precautions; provision of PPE and IPC inputs to the Bambesa, Talangai, Dembia and Mongbaya (Ganga) health centers; bio-cleaning of 2 out of 2 buildings at the Kavio health center; DLMs are only functional at the Nagbongbo, Kavio and Nakeledi health centers and triage only at the Viadana General Referral Hospital; 3 swabs carried out (Buta, Koteli and Dakwa) but no complete EDS, in a context of a strike by service providers.

### 2.4.2. Challenges.

In Bas **-Uélé,** only 130 out of 632 service providers (20.6%) are trained in IPC-WASH, 16 out of 20 priority health facilities lack functional triage, 17 out of 20 lack water for 72 hours, none of the 20 have mattresses, the available chlorine is limited to 45 kg, no WASH partners support activities around cases, and 3 complete health assessments could not be carried out. **In Tshopo,** 7 bodies were not swabbed due to non-application of the decree, none of the 595 targeted community health workers and leaders have yet been trained in IPC, and IPC supplies, access to water in health facilities, and the number of trained service providers remain insufficient. **In North Kivu,** none of the 16 decontaminated households were provided with kits, 7 out of 17 assessed PPLs are at high risk of exposure and gaps in ring opening and decontamination persist in Katwa, Butembo, Vuhovi and Beni.

## 2.5. Holistic care

### 2.5.1. Main actions

**In Ituri,** 78 new admissions, all suspected cases, were recorded, compared to 86 discharges, including 16 recoveries and 5 discharges without medical advice (1 in Biakato and 4 in Jina). Thus , 343 patients remained in isolation at the end of the day (351 the previous day), representing an overall occupancy rate of 33.8%; 332 out of 341 patients (97.4%) received [treatment/treatment].

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hot meals, all children under LNPE/SLM were taken care of (10/10 at the CTE, 4/4 in nursery and 64/64 in community), 27 patients were evacuated and 17 out of 34 transfers (50.0%) were organised, including 6 to other CTEs (2 from Mangala to the CTEs of Nizi and CME, 1 from Bafwambaya to Nia-Nia and 3 from Nia-Nia to Kisangani).

- **In North Kivu,** 82 new admissions (13 confirmed and 69 suspected) were recorded against 79 discharges, including 3 recoveries (2 in Katwa and 1 in Masereka), 72 non-cases and 4 deaths of confirmed cases (1 in Beni and 3 in Katwa). In total, 634 patients (180 confirmed and 454 suspected, including 74 children under 5 years old and 28 critically ill patients) remain in isolation at the end of the day (631 the previous day), including 233 outside CTE/CT, representing an overall occupancy rate of 62.7% (94.6% in CTE/CT; 401/424 beds); 11 out of 17 ambulances (64.7%) carried out 21 trips, of which 20 were successful (95.2%), 1,054 beneficiaries received hot meals and the 17 separated or orphaned non-breastfed infants are on SLM (100%).

- **In Tshopo,** 1 new suspected admission was recorded against 3 discharges, including 2 recoveries and 1 confirmed death (case no. 87, Kabondo Health Zone); 6 patients remain in isolation at the end of the day (8 the day before), representing an occupancy rate of 19.4%, none of whom are in serious condition, and the Kongakonga CT remains unoccupied (0/4); the 7 confirmed cases and the suspected case received hot meals, as did the 51 PPLs being monitored (100%), and the 3 available ambulances completed 2 out of 2 trips successfully.

- **In Haut-Uélé,** 7 new admissions (2 confirmed, including 1 PPL at the Bolebole CT, and 5 suspected, including 3 children under 5 years old and 1 PPL at the Pawa CTE) were recorded against 5 discharges, including 1 recovery in Isiro, 2 non-cases and 2 deaths of confirmed cases (1 in Isiro, the newborn of a recovered woman who also died, and 1 in Pawa); 33 patients remain in isolation at the end of the day (31 the day before), representing an occupancy rate of 23.9%, none of whom are in serious condition; 31 patients, 40 caregivers and 80 PPL received hot meals, 7 ambulance trips were made and the Boma Mangbetu CTE received 26 cartons of cereals from the WFP for the recovered.

**In Bas-Uélé,** no new admissions or discharges were recorded, and 40 suspected cases remain isolated in ESS facilities due to the lack of a functioning CTE, following a correction of Bondo's data (4 suspected cases actually isolated instead of 19). In Sud-Ubangi, no patients are hospitalized.

#### 2.5.2. Challenges

**In Ituri,** the blood shortage persists, 35 patients are critical, 9 patients did not receive food in Mandima, the capacity remains low in Mambasa, Kilo, Lolwa, Mandima and Biakato and the Kasenyi CTE did not report due to the strike of providers (completeness: 27/28); in North Kivu, the CTE/CTs are close to saturation (94.6%), 233 patients are being treated outside CTE/CTs, 28 patients are critical and the completeness of the PEC data is only 71% (Oicha, Vuhovi, Mabalako and Biena missing); **In Tshopo,** 2 confirmed escapees from Bafwasende remain missing, the house of a RECO was burned down in Bafwasende, the number of deaths remains high during the week and data entry is hampered by the multiplicity of templates; **in Bas-Uélé,** the standardized CTE in Buta is still under construction, no ambulance is available and none of the 40 isolated suspects are receiving nutritional support; in Haut-Uélé, a strike by service providers is threatening the CTEs, only 2 out of 8 CTEs are standardized, network coverage remains weak in some health zones and nutritional inputs (F75, F100, Plumpy'Nut) as well as the water and energy supply to the CTEs remain insufficient.

## 2.6. Vaccination

### 2.6.1. Main actions

- **In Ituri,** 133 people were vaccinated during the day (53 women, including 5 breastfeeding women, and 80 men), bringing the total to 2,699 people vaccinated (1,137 women, including 27 pregnant women and 80 breastfeeding women, and 1,562 men), with 35 non-severe AEFI and 5 severe AEFI reported since the beginning; the TUMAINI, Partners, EBO-PEP and Oracle clinical trials as well as qualitative and operational surveys are continuing.

  **In North Kivu,** 306 PPLs were vaccinated by 2 teams supervised in the field (126 at the Mabalako General Referral Hospital and 180 at the Kanzuli Health Center, Beni Health Zone), bringing the total to 1,617 vaccinated (882 men and 735 women, including 12 breastfeeding women) out of 7,105 expected (22.8%), with 983 doses remaining out of 2,600 received, no doses lost and 1 non-serious AEFI reported.

- **In Bas-Uélé,** the deployment of a compassionate vaccination support team is being prepared in the Bili health zone, pending central-level agreement for Bondo and Monga, and pre-registration of PPL and TPL continues; cumulatively, 2,000 people out of 13,202 targeted (15.1%) have been vaccinated in the province.

### 2.6.2. Challenges

**In Ituri,** the TPL/PPL target remains to be redefined, and 5 serious AFI incidents have been reported cumulatively; in the North-In Kivu, vaccination remains concentrated in two health zones (Beni: 92.2% vaccinated; Mabalako: 7.8%) and mistrust among some leaders in Butembo, linked to the delayed availability of the vaccine,

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persists; in Bas-Uélé, the start-up of the UCCs is still not effective, the support of field actors is lagging behind and the ZS of Bili still has no vaccinated people (0/1,067).

### 2.7. Continuity of Care

#### 2.7.1. Main actions • In North Kivu, the

baseline assessment of the Graben University Clinic revealed triage respecting the circuit, a maternity ward where 100% of providers are trained in SONU B and CPNr and nutritional care provided with available inputs, but an insufficiency of PPE, pedal bins and hydroalcoholic solution, non-standardized emergency services located 50 m from other services, non-integrated tuberculosis management, unavailability of MEGs and only 44 out of 240 staff (18.3%) trained in IPC; at the Matanda secondary hospital, the presence of a neuropsychiatrist, a psychologist and a medical ambulance as well as a 5-minute waiting time contrast with the absence of an updated referral card, a dashboard for monitoring service disruptions and an absenteeism plan.

- **In Tshopo,** 4 health centers were evaluated (CM St Luc in Makiso-Kisangani, CM Jéhovah Jiré in Mangobo, CM Odile and CM La Source in Tshopo): CM St Luc has an isolation room and its 13 staff members are vaccinated (100%), but 15 out of 38 children and 8 out of 22 pregnant women missed their vaccination appointments; at CM Jéhovah Jiré, 6 out of 7 staff members (86%) are vaccinated and there is no triage area; 9 out of 9 providers are trained on Ebola at CM Odile, which however does not have any functional incinerators, compared to 2 out of 10 at CM La Source; The recovery plans for CM La Famille and HGR Kabondo have been developed, training for managers and service providers on transfusion safety has begun and 2 recovered patients have been transferred from CTE to HGR Kabondo.

- **In Haut-Uélé,** the CREC supervised the integration of Ebola messages into the distribution of mosquito nets in the health districts of N'Sele, Gossamu and Mayogo; the continuity of care pillar did not transmit any figures for the day.

#### 2.7.2. Challenges

Implementation of the recovery plans for health and social care facilities assessed in Tshopo is still pending, where the strike by public health and social care workers, the non-activation of the Expanded Programme on Immunization (EPI) continuity plan, the non-use of the partogram and the absence of up-to-date birth registers persist, with a decline in the use of curative services, IMCI, ANC, maternity and vaccination services in the health districts of Malkia and Banduku; in North Kivu, PPE, MEG and IPC-trained staff remain insufficient at the Graben University Clinic, and the Matanda secondary hospital, accessible by several paths conducive to patient escapes and attacks on providers, has no contingency plan for absenteeism; no data transmitted by Ituri, Haut-Uélé, Bas-Uélé, Sud-Ubangi and Nord-Ubangi.

## 2.8. Risk Communication and Community Engagement (RCCE)

### 2.8.1. Main actions

- **In Ituri,** 21,922 home visits reached 80,420 people, 618 educational talks brought together 6,560 people, 53 mass awareness campaigns reached 1,435 people, 10 community debates reached 217 people and 6 advocacy campaigns reached 11 religious authorities; 402 community alerts, including 191 deaths, were notified, 29 feedbacks out of 36 (80.6%) and 15 incidents out of 17 (88.2%) were resolved within 48 hours; The CREC facilitated the pre-listing of 972 contacts around 6 confirmed cases, the decontamination of 26 out of 26 households, including 2 structures, and of 2 out of 2 ESS, the completion of 30 out of 30 EDS in the AS of Bankoko (Bunia), 3 megaphones, 1 carton of hydroalcoholic gel and 50 posters on the community definition of cases were given to the avenue chiefs and a handwashing kit to the motorcycle taxi committee.

In North **Kivu,** 17,450 home visits reached 55,763 people, 147 educational talks reached 3,207 people, and 43 mass communication sessions 2,867 people and 3 briefings 97 people; 567 alerts out of 650 expected (87.2%) were notified, 16 feedbacks out of 23 (69.6%) and 6 incidents out of 15 (40.0%) resolved within 48 hours and 3,153 people out of 4,750 (66.4%) engaged in community actions; 134 leaders of Revival churches in the Vungi AS (Katwa) were sensitized on risky practices, religious leaders and the Union of Young Catholics of Manguredjipa were mobilized, advocacy was carried out with the Mwami of Baswagha (Musienene) against clandestine burials and 50 local leaders of the Rwangoma AS (Beni) were involved in a dialogue on rumors.

- **In Haut-Uélé,** 281 home visits reached 3,111 people, 27 educational talks reached 192 people and 10 mass activities reached 3,087 people, for a total of 6,390 people sensitized; 9 community alerts (1 in Isiro and 8 in Wamba) and 1 death alert were notified and 13 feedbacks out of 13 (100%) were resolved within 48 hours; 31 people linked to 2 community deaths were sensitized to contact tracing, decontamination and EDS (AS of Tely and ZS of Boma Mangbetu), Gossamu and Mayogo as well as joint passenger sensitization to PoCs.

- **In Tshopo,** approximately 12,561 people (4,536 men, 5,008 women and 3,017 students) were sensitized, notably during 9 mass awareness sessions (Lubunga, Kabondo, Makiso, Bafwasende and

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Tshopo), 4 educational talks (314 people) and in the Lisanga and Ndolenge EP (AS of Pumuzika); 6 programs were broadcast on RTNC, RFO, Canal Orient, Ben Ezer and Nuuru, 21 members of the NGO LVS were trained, the PRECODESA of the Tshopo ZS were strengthened on the listing and follow-up of contacts, 118 posters and 134 advice cards were distributed, 3 out of 3 refusals were lifted and 3 out of 3 feedbacks were resolved.

**In Bas-Uélé,** 667 home visits reached 3,511 people, one educational talk reached 4 people, one community debate reached 84 people, and four mass awareness campaigns reached 1,366 people. Sixty-four community alerts were reported and resolved. CREC supported surveillance in Poko, Likati, and Dakwa and facilitated a community dialogue with the village of Abandonné 2 (Bambili group). The provincial focal point was provided with a computer by UNICEF. Only 5 out of 13 health zones reported. No CREC data was transmitted by Nord-Ubangi.

## 2.9. Mental Health and Psychosocial Support (MHPSS)

### 2.9.1. Main actions

- **In Ituri,** 17 confirmed cases out of 31 (54.8%), 66 confirmed out of 86 and 114 suspected out of 179 under follow-up as well as 34 new suspected out of 77 were taken care of, 13 households of separated and orphaned children out of 17 (76.5%) were accompanied, 79 children followed, including 19 out of 19 in nursery with 1 new admission, 14 recovered out of 16 reintegrated (87.5%) and 48 out of 52 followed in community, and 60 results announced, including 17 positive; 192 accompanying persons and 73 PPLs were supported.

- **In North Kivu,** 8 out of 8 notifying health zones (100%) were covered, 29 out of 39 new cases (74.4%), 216 confirmed cases under follow-up, 164 new suspected cases and 245 suspected cases under follow-up were taken care of, 23 out of 30 (76.7%) separated and orphaned child households were supported, 6 out of 6 children were followed up in daycare and 28 out of 36 (77.8%) in the community, 9 recovered patients were reintegrated and 11 out of 11 were followed up in the community, 101 results were announced, including 39 positive results, 422 people (133 men, 207 women and 82 children) benefited from psychoeducation (UNICEF support), 89 households were supported for public health actions and 67 PPLs were supported; The training of members of the SMSPS pillar in the Kyondo health zone was supported by the WHO. In Haut-Uélé, no SMSPS data was transmitted for the day.

- **In Tshopo,** the 6 notifying health zones were covered (100%), the last confirmed case (1/1) and the 6 confirmed cases under follow-up were taken care of, as well as the new suspect of the day and the suspect under follow-up, the 7 confirmed cases at the CTE Météo received psychological support, 1 negative result was announced, 424 people (including 310 students from Lubunga and 40 accompanying persons at the CTE Météo) benefited from psychoeducation on barrier gestures, 2 households were supported, 1 PPL supported and the recovered person followed in the community was seen (1/1).

### 2.9.2. Challenges

**In Ituri,** only 15 out of 28 affected health zones (53.6%) are covered, barely 17 of the 31 most recent confirmed cases (54.8%) have received care, and coverage remains incomplete around the cases in Mandima (4/11), Mambasa (4/8), Mongbwalu (2/3), and Bunia (3/4). In North Kivu, 10 of the 39 new cases (25.6%) have not received support, 7 out of 30 households with separated and orphaned children remain without assistance, and 8 out of 36 children are not receiving community follow-up. In Tshopo, community health workers in the Wanie-Rukula health zone are not trained, and the health pillar lacks communication equipment and motorcycles.

## 2.10. Logistics

### 2.10.1. Main actions

- **In Ituri,** 27 patients were evacuated and 17 out of 34 transfers (50.0%) were organized by ambulances, but reporting of ambulance movements remains low; supplies: 3 megaphones, 1 carton of hydroalcoholic gel, 50 posters and 1 handwashing kit given to community relays of the Bankoko health sector; for health products and medicines (HPM), the blood shortage persists, IPC inputs remain insufficient in Mandima, Mambasa, Nia-Nia and Nizi and the installation of 3 laboratories is planned in the health zones of Fataki, Bambu-Mine and Mangala.

- **In North Kivu,** 11 out of 17 ambulances (64.7%) were mobilized for 21 trips, 15 schools in Mabalako are being supplied with PCI-WASH kits by NRC and, for the PSM, 2,600 doses of Ervebo have been received cumulatively (1,617 used, 983 remaining, none lost), while the laboratory expresses needs for RNA extraction kits, Altona PCR kits, plate centrifuge and Starlink router.

- **In Tshopo,** the tools of the alert centre (9 telephones for the DPS and 8 ZS and 5 computers for the DPS and 4 ZS) were received and deployed, the CTE Météo was supplied with medical inputs, the premises were freed up for the extension of the standardized CTE Météo and the rehabilitation of the isolation of Makiso, and the mobility of the pillars as well as the transport of the participants to the RIA were ensured; the 3 available ambulances were mobilized (100%) for 2 successful trips out of 2 and the need for mobility of the ZS is estimated at 115 motorcycles.

- **In** Haut-Uélé, a 45 KVA generator was received in Isiro for the logistics and coordination base (WFP support), 1,400 liters of fuel were received in Pawa (MSF Netherlands), and response supplies were

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Received at Boma Mangbetu, 2 vehicles and 1 generator were supplied with fuel (FHI 360), the ambulance of the Isiro Health Zone was repaired and the PoC PK10 Poko Road was equipped with equipment; for the PSM, nutritional inputs (F75, F100, Plumpy'Nut) are lacking in the CTEs and PCI kits are missing for the ESS, while mobility (vehicles and motorcycles), the fencing of the UMS and the ventilation of the depot remain to be ensured.

**In Bas-Uélé,** the Bondo Health Zone has been supplied with EDTA tubes for sampling, the packaging of PCI and PEC supplies for Bondo is planned, the rehabilitation of the meeting room and offices of the Provincial Health Division (DPS) continues with the WHO, and the construction of the Ebola Treatment Center (ETC) at the Buta General Referral Hospital (HGR) is 50% complete; for the Medical Support Centers (PSM), the chlorine stock is limited to 45 kg, the laboratory has no fire extinguisher, uninterruptible power supply (UPS), or computer, the generator is broken, no diesel fuel is available, and the 20 priority ESSs (Emergency Social Services) lack mattresses. No logistical data has been transmitted by Nord-Ubangi.

### 2.11. Security

## 2.11.1. Main actions

- **In Ituri,** the Avakubi PoC remains non-functional due to insecurity; in North Kivu, EDS teams were accompanied and secured for the burial of 2 confirmed cases at the Bundji cemetery, the swab and burial of a body in Bwasa and the swab of a body at CM La Brèche, while the ambulance of the Oicha General Referral Hospital was attacked with stones on November 2; **in Haut-Uélé,** the security threat against PoC providers persists on the Bolebole axis (Pawa and Wamba); in Tshopo, threats and attacks against response teams persist, especially in Bafwasende, Kabondo and Mangobo; in Bas-Uélé, the security situation was assessed with the partner Palladium in the context of a strike by providers and the response was secured at the zonal level.

## 2.12. PSEA (Prevention of Sexual Exploitation and Abuse)

## 2.12.1. Main actions

**In Ituri,** 39 PoE providers in the Mahagi health zone (12 women and 27 men) were briefed on the PSEAH and signed the code of conduct, 62 people (choir members from the Ndibe catechism and women beneficiaries of the CPS) were sensitized in Bunia and the display of messages and the distribution of leaflets continue in the health zones of Bunia, Aru, Mongbwalu, Mambasa, Fataki and Mandima, with 3 partners reporting (WHO, IMC and IOM)

**In North Kivu,** a PSEA coordination meeting brought together 12 participants from the Red Cross, UNICEF, FHI 360, HEAL Africa, and IMC. Seventy-three community health workers and local leaders were briefed in Alimbongo, and awareness-raising sessions on PSEAH and reporting via the 495555/151 hotline reached 58 traders in the Kikwaso gallery (61 leaflets distributed), 291 students from the Hodari Institute in Beni, 176 students from the Mwikwa Don Bosco school and 31 teachers from the Kavaghendi Institute in Butembo, and 231 participants from the Kitwadi Primary School in Walikale, with support from the WHO.

**In Tshopo,** 32 members of pressure groups were sensitized on the PSEAH, the referral circuit and zero tolerance (13 in Kabondo and 19 in Mangobo), the mapping of hotels in Makiso was developed and the schedule of PSEAH activities was agreed upon with the IMC; in Haut-Uélé, 18 people were sensitized on the PSEA, referral mechanisms, the reporting circuit and the rights of victims (5 community members in Boma and 13 Caritas agents in Wamba, AS of Maman Kahenga), bringing the total to 3,784 people sensitized in the community.

### 2.12.2. Challenges

- Coverage of affected areas remains low: only Ituri, North Kivu, Haut-Uélé and Tshopo reported their PSEA activities for the day (4 out of 8 DPS; 50.0%), with North Kivu and Haut-Uélé having only one reporting partner (WHO, out of 14 expected in Haut-Uélé); the reporting mechanism is known in only 15 out of 36 health zones (41.7%) in Ituri, 6 out of 13 health zones (46.2%) in Haut-Uélé and 4 out of 34 health zones (11.8%) in North Kivu, and the community complaint management mechanism remains to be installed in 3 other health zones from Kisangani to Tshopo; awareness-raising tools, visibility materials and IT kits for the pillar remain insufficient and the mapping of stakeholders is not controlled, with some partners refusing alignment.

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## KEY MESSAGES

Sixty-two (62) new confirmed cases were reported in four provinces, including 36 in North Kivu, 23 in Ituri, and 3 in Haut-Uélé (2 in Isiro and 1 in Wamba). Transmission remains particularly high in the Beni health zone (22 cases) in North Kivu.
Kivu and in those of Mandima (9 cases), Mambasa (5 cases) and Mangala (4 cases)
in Ituri.

Of the 33 confirmed deaths reported, 17 were community deaths (10 in North Kivu and 7 in Ituri) and 16 were deaths in Ebola Treatment Centers (9 in Ituri, 4 in North Kivu, 2 in Haut-Uélé, and 1 in Tshopo). The contact tracing rate was 83.9% (33,281/39).
667) in the five provinces with contacts currently being monitored (South Ubangi: 97.3
%; Ituri: 88.9%; Haut-Uélé: 84.8%; North Kivu: 79.6%; Tshopo: 65.3%), in
a slight decrease compared to the previous day (84.4%) and still below the 95% threshold, with the lowest rate in Tshopo, where the health workers' strike is disrupting the
follow-up. ; **In Bas-Uélé, all contacts have completed their follow-up period.**
**However, six (6) contacts have been lost to follow-up for more than 27 days, without that their status.**

## TIMELINE OF KEY EVENTS

From April 24 to May 15, 2026, the epidemic passed from the detection of the first suspected cases to the Biological confirmation of the Ebola Bundibugyo virus leading to the official declaration of the 17th
Ebola Virus Disease Outbreak in the Republic
Democratic Republic of Congo.

Between May 17 and 18, 2026, the authorities national authorities, with the support of partners, have strengthened the response by declaring a state of emergency. public health of national scope then
continental, accompanied by the mobilization of funds.

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EBOLA PRESIDENTIAL FORCE TASK 17

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**National Institute of Public Health**

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